

APPROVED

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STATE PROGRAM "People's Health and
Demographics"
security of the Republic of Belarus" on
2016 – 2020

CHAPTER

1 GENERAL PROVISIONS

State program "Health of the people and demographic security of the Republic of Belarus" for 2016-2020 (hereinafter referred to as the State Program) was developed in accordance with the priority directions of socio-economic development defined in National Strategy for Sustainable Socioeconomic Development of the Republic of Belarus for the period up to 2020 and specified in the National strategies for sustainable socio-economic development of the Republic Belarus until 2030, and also taking into account such areas of the Program socio-economic development of the Republic of Belarus for 2016-2020, as "Improving the demographic potential and strengthening the institution families" and "Improving the health level of Belarusian citizens and accessibility medical services".

In these documents, the transition to sustainable development of the republic associated with improved physical and reproductive health population, creating preconditions for increasing the birth rate and increasing life expectancy, comprehensive strengthening of the institution families, and the stabilization of the population and an increase in the expected life expectancy is one of the goals of sustainable development in the medium term (2016 – 2020).

The state program provides for the creation of conditions to improve the health of the population, covering all stages of life, a wide awareness of the population about risk factors that threaten health, promotion of a healthy lifestyle, formation of awareness among the population self-preservation behavior, development and strengthening of family values, increasing the prestige of parenthood, improving population reproduction, reducing mortality rates, especially in the working age group, increasing life expectancy, improving the quality and

accessibility of healthcare services, as well as optimization
internal migration processes.

CHAPTER 2

STRUCTURE AND CUSTOMERS OF THE STATE PROGRAM

The state program includes the following subprograms: subprogram 1
"Family and childhood"; subprogram 2
"Prevention and control of non-communicable diseases"
diseases";
subprogram 3 "Prevention and overcoming of drunkenness and
alcoholism";
subprogram 4 "Tuberculosis";
subprogram 5 "Prevention of HIV infection";
subprogram 6 "External migration";
subprogram 7 "Ensuring the functioning of the system
of Healthcare of the Republic of Belarus".

The Customer - Coordinator of the State Program is
Ministry of Health.

Customers of subprogram 1 "Family and Childhood" – Ministry
Health, Ministry of Labor and Social Protection, Ministry
Education, Ministry of Emergency Situations, Ministry
architecture and construction, Ministry of Internal Affairs, Ministry
information, Ministry of Culture, Ministry of Finance, National

State Television and Radio Broadcasting Company, regional executive committees, Minsk City Executive Committee.

Customers of subprogram 2 "Prevention and control of non-communicable diseases
diseases" - Ministry of Health, Ministry of Internal Affairs
affairs, Ministry of Housing and Public Utilities, Ministry
information, Ministry of Culture, Ministry of Education, Ministry of Taxes and
Duties, Ministry of Emergency Situations
situations, the Ministry of Agriculture and Food, the Ministry of Sports and
Tourism, the Ministry of Trade, the Ministry
Labor and Social Protection, Ministry of Economy, State
Standardization Committee, State Customs Committee, Belarusian State Food
Industry Concern

"Belgospischeprom", National State Television and Radio Broadcasting Company, National
Academy of Sciences of Belarus, regional executive committees, Minsk City Executive Committee.

Customers of subprogram 3 "Prevention and overcoming of drunkenness"
and alcoholism" - Ministry of Health, Ministry of Internal Affairs
affairs, Ministry of Information, Ministry of Culture, Ministry
Education, Ministry of Taxes and Duties, Ministry of Agriculture
economy and food, Ministry of Sports and Tourism, Ministry

trade, Ministry of Transport, Ministry of Labor and Social protection, Ministry of Economy, Ministry of Finance, State Standardization Committee, State Border Committee, State Customs Committee, Belarusian State food industry concern "Belgospisheprom", National State Television and Radio Broadcasting Company, Belarusian Republican Union consumer societies, regional executive committees, Minsk City Executive Committee.

Customers of subprogram 4 "Tuberculosis" – Ministry health, the Ministry of Internal Affairs, regional executive committees, Minsk city executive committee.

The customers of subprogram 5 "HIV infection prevention" are the Ministry of Health, the Ministry of Information, the Ministry of Education, the Ministry of Emergency Situations, regional executive committees, the Minsk City Executive

Committee. The customers of subprogram 6 "External Migration" are the Ministry of Internal Affairs, Ministry of Foreign Affairs, Ministry of Information, Ministry of Education, Ministry of Labor and Social Protection, Ministry of Economy, Ministry of Finance, State Border Committee, National Academy of Sciences of Belarus, National State Television and Radio Broadcasting Company, regional executive committees, Minsk City Executive Committee.

Customers of subprogram 7 "Ensuring the functioning of the system of Healthcare of the Republic of Belarus" – Ministry of Healthcare, Ministry of Internal Affairs, Ministry of Defense, Ministry of Sports and Tourism, Ministry of Labor and Social Protection, State Border Committee, Office of the President of the Republic Belarus, National Academy of Sciences of Belarus, Republican State and public association "Belarusian physical culture and sports society "Dynamo", regional executive committees, Minsk city executive committee.

CHAPTER 3

PURPOSE AND OBJECTIVES OF THE STATE PROGRAM

Realization of the constitutional rights of citizens to health protection, a favorable environment, protection of the family, motherhood, fatherhood and childhood is the most important task of the state's social policy. One of the priorities of state policy is the creation of conditions for the qualitative development of human potential, healthy and decent life for the population, including on the basis of the implementation of a complex measures to improve health, increase fertility and expected life expectancy.

Based on the stated priorities, the goal of the State Program are the stabilization of the population and the increase in expected life expectancy.

To achieve the set goal, a solution is envisaged the following tasks.

The objectives of subprogram 1 "Family and Childhood" are: improving the system of protecting the health of mothers and children; developing a system of support for families with children and improving conditions their life activities, strengthening the institution of the family; ensuring the rights and legitimate interests of children. The implementation of these tasks will be carried out by implementation of activities of subprogram 1 "Family and Childhood" in accordance with Appendix 1.

The objectives of subprogram 2 "Prevention and control of non-communicable diseases" are: reducing the

impact of risk factors for non-communicable diseases by creating a unified preventive environment; ensuring the prevention of non-communicable diseases throughout the life cycle through universal and accessible coverage of the population with primary health care services; reduction of premature mortality and stabilization of disability population, caused by non-communicable diseases; ensuring monitoring of the health status of the population through the creation of a single information space of health care of the Republic of Belarus. These

tasks will be solved through the implementation of measures subprogram 2 "Prevention and control of non-communicable diseases" according to Appendix 2. The objectives of subprogram

3 "Prevention and overcoming of drunkenness" and alcoholism" are:

reducing the level of negative social and economic consequences of drunkenness and alcoholism; reduction in the volume of alcohol consumption by the population, low-alcohol drinks, beer.

The implementation of these tasks will be carried out through the implementation activities of subprogram 3 "Prevention and overcoming of drunkenness" and alcoholism" according to Appendix 3. The

objectives of subprogram 4 "Tuberculosis" are: prevention of mortality from tuberculosis; prevention of tuberculosis morbidity; provision of high-quality treatment for patients with multiple drug-resistant forms of tuberculosis.

The specified tasks are planned to be solved by implementing measures subprogram 4 "Tuberculosis" in accordance with Appendix 4.

The objectives of subprogram 5 "Prevention of HIV infection" are: ensuring universal access to diagnosis, treatment, care and social support in connection with HIV infection, including penitentiary system;

elimination of vertical transmission of HIV infection from mother child and cases of HIV transmission associated with the provision of medical care assistance;

reducing the spread of HIV in high-risk populations risk of infection. These

tasks will be solved through the implementation of measures subprogram 5 "Prevention of HIV infection" in accordance with Appendix 5.

The objectives of subprogram 6 "External Migration" are: managing external migration taking into account demographic and socio-economic development of the country's regions, as well as the return compatriots;

improving the system of detection, prevention and suppression illegal migration, as well as the integration of foreigners;

improving the procedure for attracting foreign specialists to the Republic of Belarus, as well as counteracting illegal integration. The implementation of these tasks will be carried out through the implementation activities of subprogramme 6 "External Migration" in accordance with Appendix 6.

The tasks of subprogram 7 "Ensuring the functioning of healthcare system of the Republic of Belarus" are:

creation of conditions for the dynamic development of healthcare; provision of conditions for the provision of services by organizations health care in accordance with their statutory activities.

These tasks will be solved through the implementation of measures subprogram 7 "Ensuring the functioning of the health care system Republic of Belarus" in accordance with Appendix 7.

CHAPTER

4 TARGET INDICATORS OF THE STATE PROGRAM

The consolidated target indicator of the State Program and the target The indicators of the subprograms according to Appendix 8 provide for to ensure by 2020: an

increase in life expectancy to 74.6 years; no infant mortality rate of more than 3.4 ppm per 1000 live births;

reduction of child mortality at the age of 0-18 years to 40.5 procentimille per 100,000 people; an increase in the total fertility rate to 1.75; a decrease in the prevalence of tobacco use among people in aged 16 years to 24.5 percent; increase in physical activity of the adult population (average physical activity of at least 30 minutes a day) up to 40 percent; reducing daily consumption of table salt to 5 grams; reducing the content of trans fatty acids in foods processing of vegetable oils to 2 percent or less; the share of doctors working on the principle of general practitioners, in the total number of doctors working in the primary care system medical care – 100 percent; the proportion of patients with malignant tumors in stages I – II diseases in the total number of patients with identified in the process screening for malignant diseases – 70 percent; coverage of rehabilitation of patients of working age after myocardial infarction, acute cerebrovascular accident, surgical interventions for malignant, neurosurgical, traumatological and other non-infectious diseases - not less than 80 percent; reduction of the mortality rate of the working-age population to 3.8 ppm per 1000 people; an indicator of the severity of the initial transition to disability of persons working age – 50 percent; reduction in mortality from accidental alcohol poisoning to 10.6 centimille per 100 thousand people; reduction of alcohol consumption per capita to 9.2 liters; reduction of tuberculosis incidence in the population to 30.6 cases per 100 thousand people; reduction of mortality from tuberculosis to 4.41 cases per 100 thousand people; the proportion of patients with multidrug-resistant forms of tuberculosis who successfully completed the full course of treatment (18–24 months), in the total number of such patients – not less than 60.4 percent; Coverage of combination antiretroviral therapy among HIV-positive people patients requiring treatment – 80 percent; reduction in the risk of HIV transmission from an HIV-infected mother child up to 1 percent; coverage of population groups with high risk of infection HIV prevention measures up to 50 percent;

the proportion of completed volumes of medical care provided by type in volumes approved by plans for the provision of medical care assistance – 90 percent;

migration increase – 70 thousand people. Information

on the procedure for collecting information and the methodology for calculating the consolidated target indicator of the State program and target indicators subroutines are defined according to Appendix 9.

CHAPTER 5

FINANCIAL SUPPORT OF THE STATE PROGRAM

Financial support for the State program in accordance with Appendix 10 will be implemented at the expense of the republican and local budgets, other sources not prohibited by law, with annual clarification of funding volumes during formation republican and local budgets for the next financial year.

The implementation of the State Program will require funds in the amount of 274,717,166.1 million rubles, including funds from the republican budget – 58,534,064 million rubles, of which state funds targeted budget funds (preventive funds) – 743,548.5 million rubles, local budgets – 201,273,514 million rubles, own funds organizations – 14,497,355.6 million rubles, other sources – 412,232.5 million rubles. The volume of funding for the activities of subprogram 1 "Family and childhood" amounts to 12,503,328.7 million rubles, including funds republican budget – 12,034,288.8 million rubles, local budgets – 456,316.1 million rubles, other sources (funds of the Organization Fund United Nations Population Fund (hereinafter referred to as UNFPA), the United Nations Children's Fund (hereinafter referred to as UNICEF) - 12,723.8 million rubles. To finance the

activities of subprogram 2 "Prevention and "control of non-communicable diseases" will be allocated 3,915,930.2 million rubles, including funds from the republican budget - 1,622,110.6 million rubles, local budgets - 2,232,696.6 million rubles, and other sources (funds from the World Health Organization (hereinafter referred to as WHO), UNICEF, UNFPA, United Nations Development Program (hereinafter referred to as UNDP) – 61,123 million rubles.

The volume of funding for the activities of subprogram 3 "Prevention and overcoming drunkenness and alcoholism" amounts to 392,095 million rubles, in including funds from the republican budget – 103,100 million rubles, local budgets – 288,995 million rubles.

RUB 1,212,257.5 million will be allocated to finance the activities of subprogram 4 "Tuberculosis", including funds

republican budget – 493,780.3 million rubles, of which funds state target budget funds (preventive funds) – 1100 million rubles, local budgets – 573,090 million rubles, other sources (United Nations Global Fund for fight against AIDS (hereinafter referred to as the Global Fund) – 145,387.2 million rubles.

The volume of funding for the activities of subprogram 5 "Prevention HIV infection" amounts to 908,895.9 million rubles, including funds republican budget – 509,361.9 million rubles, local budgets – 206,535.5 million rubles, other sources (funds from the Global Fund, the Joint United Nations Programme on HIV/AIDS (hereinafter referred to as UNAIDS) – 192,998.5

million rubles. Financing of the activities of subprogramme 6 "External Migration" will be carried out at the expense of the republican budget in the amount of 35,427.6 million

rubles. The volume of financing for the activities of subprogram 7 "Provision functioning of the healthcare system of the Republic of Belarus" amounts to 255,749,231.2 million rubles, including funds from the republican budget – 43,735,994.8 million rubles, of which state funds targeted budget funds (preventive funds) – 742,448.5 million rubles, local budgets – 197,515,880.8 million rubles, own funds organizations – 14,497,355.6 million rubles.

CHAPTER 6 ANALYSIS OF THE MAIN RISKS IN IMPLEMENTATION STATE PROGRAM

When implementing the State Program, the impact on the achievement The established indicators can have the following effect:

environmental risks associated with natural and man-made disasters. To minimize the impact of this group of risks

It is planned to implement continuous monitoring of natural and weather conditions conditions in the country, the state of production, the implementation of preventive measures measures to avoid natural and man-made disasters;

medical risks associated with disease epidemics. Overcoming these risks is possible through the implementation of large-scale preventive measures, timely adoption measures aimed at localizing epidemics, implementing permanent control over the level of morbidity of the population;

socio-demographic risks associated with a decrease in the number women of reproductive age, increasing the age of women at the birth of the first child, families planning more children late term. Minimization of the above risks is possible by carrying out

explanatory work among the population, starting from adolescence, on issues of family planning, ensuring socio-economic stability in society, raising the educational level of individuals reproductive age and provision of social assistance to young families; medical and social risks associated with early development bad habits, unplanned pregnancy, presence of diseases women with a hereditary burden. This risk group can only be eliminated or minimized through large-scale preventive work with the population to develop motivation for healthy lifestyle, fight against bad habits, provided attracting broad sections of the population to physical education, tourism, cultural events and raising the level of education;

legal risks associated with changes in legislation, the duration of the period of formation of the regulatory framework necessary for the effective implementation of the State Program. To minimize the impact of this group of risks at the development stage

of regulatory legal acts is planned to be involved in their discussion stakeholders who must subsequently take action participation in their coordination, monitoring of planned changes in legislation;

financial risks may be caused by insufficient volumes financing, reduction of the funds provided for the implementation events. Such risks can be overcome by annual clarification of the amounts of financial resources allocated for implementation activities of the subprograms of the State Program, depending on achieved results, defining priorities for the first financing;

organizational risks associated with ineffective management implementation of subprograms of the State program, interdepartmental interaction, shortage of qualified personnel. The impact of the above-mentioned risks may result from failure to fulfill the tasks of the State subprograms programs, reducing the planned values of its indicators, efficiency and the quality of the implementation of subprogram activities. The main conditions minimization of organizational risks is the formation of an effective systems for managing the implementation of subprograms, increasing interdepartmental interactions during their implementation, prompt response to identified deficiencies in management, control and personnel procedures, timely adjustment of activities of the State subprograms programs. The

impact of the above risks may lead to: an increase in the infant mortality rate of over 3.4 per thousand per 1000 live births;

to a decrease in the birth rate and the total fertility rate
 birth rate below 1.75; to a
 decrease in life expectancy at birth
 below 74.6 years;
 to an increase in the level of per capita consumption of alcoholic beverages
 population over 9.2 liters; to an
 increase in mortality from tuberculosis over 4.41 cases per
 100 thousand people;
 to a decrease in the coverage of combination antiretroviral therapy
 below 80 percent of HIV-infected patients; to a decrease in
 migration growth below 70 thousand people.

CHAPTER 7

METHODOLOGY FOR ASSESSING THE EFFECTIVENESS OF THE STATE PROGRAM

The main sources of obtaining information on the implementation of targets
 The indicators of the State Program are statistical, accounting and financial reporting,
 as well as administrative
 information.

Evaluation of the effectiveness of the implementation of the State program
 is carried out annually. The results of such an assessment are presented as part of
 annual report of the Ministry of Health on the implementation of
 State program. Assessment of

the current effectiveness of the implementation of the State
 programs are produced by comparing the actual results achieved
 values of the consolidated target indicator for the relevant year with
 the consolidated target indicator value approved for the year.

Evaluation of the effectiveness of the implementation of the State program
 is carried out in five stages. At the first

stage, the degree of solution of the problems of a separate
 subroutines by formula

$$SD_{mp} = \frac{\sum_{i=1}^N SD_{i/ppz}}{N}$$

where $SD_{i/ppz}$ is the degree of solution of the tasks of a separate subprogram in the reporting year;

$SD_{p/ppz}$ – degree of achievement of the planned target value
 an indicator characterizing the tasks of a separate subprogram;

N – the number of target indicators characterizing the tasks
 a separate subroutine.

The degree of achievement of the planned value of the target indicator characterizing the task of the subprogram is calculated according to the following formulas:

for target indicators, the desired trend of which is increase in values:

$$SDp/ppz = ZPp/pf / ZPp/pp,$$

for target indicators whose desired trend is decrease in values:

$$SDp/ppz = ZPp/pp / ZPp/pf,$$

where SDp/ppz is the degree of achievement of the planned value of the target indicator characterizing the tasks of the subprogram;

$\ddot{y}\ddot{y}/\ddot{y}\ddot{y}$ – the value of the target indicator characterizing the task subprograms actually achieved at the end of the reporting period;

$\ddot{y}\ddot{y}/\ddot{y}\ddot{y}$ – planned value of the target indicator characterizing subroutine task. If the value of

SDp/PPz is greater than 1, then when calculating the degree of solution for tasks of a separate subroutine it is taken equal to 1.

At the second stage, the degree of goal achievement is assessed. State program. To assess the

degree of achievement of the goal of the State program the degree of compliance with the consolidated target indicator is determined State program planned value according to the formula

$$SDgppz = ZPgpf / ZPgpp,$$

where $SDgppz$ is the degree of achievement of the planned value of the consolidated target indicator characterizing the goal of the State Program;

$\ddot{y}\ddot{y}\ddot{y}\ddot{y}$ – the value of the consolidated target indicator characterizing the goal of the State Program, actually achieved by the end reporting period; $\ddot{y}\ddot{y}\ddot{y}$ –

planned value of the target indicator characterizing the goal of the State Program.

If the value of $SDgppz$ is greater than 1, then when calculating the degree of achievement planned value of the consolidated target indicator characterizing the goal of the State Program, it is taken equal to 1.

At the third stage, the average level of achievement is assessed goals of the State program and solutions to the tasks set subroutines by formula

$$\text{They are } Mr = \frac{SD_{gppz} + \sum_{i=1}^n SR_{n/n_i}}{8},$$

where SUGP is the average level of achievement of the State Goal programs and solutions to the assigned tasks of subprograms; SDgppz – the degree of achievement of the planned value of the consolidated target indicator characterizing the goal of the State Program; SRp / – the degree of solving the problems of a separate subprogram. If the value of SUGP is greater than 1, then when calculating the average level the degree of achievement of the goal of the State program and decision for the assigned tasks of subroutines it is taken to be equal to 1.

At the fourth stage, the effectiveness of the implementation of a separate subroutines as a ratio of the degree of solving problems of a separate subprograms to the degree of compliance with the actual volume of funding of this same subprogram to its planned volume in the reporting year formulas

$$Ep/p = SRp/p / SSuz,$$

where Ep/p is the efficiency of implementing a separate program in the reporting year; CPÿ/ÿ – the degree of solution of tasks of a separate subprogram in the reporting year; ÿÿ/ the degree of compliance with the actual volume of financing subprograms to its planned funding volume in the reporting year.

At the fifth stage, the effectiveness of implementation is determined State program as a ratio of the average level of degree achieving the goals of the State Program and solving the problems tasks of subroutines to the degree of compliance with the actual volume financing of the State program in the planned volume financing in the reporting year according to the formula

$$Egp = SUGp / SSuz,$$

where Egp is the efficiency of the implementation of the State Program; SUGP – average level of achievement of the State goal programs and solutions to the tasks set by subprograms; SSuz - the degree of compliance with the actual volume of funding The state program for its planned volume of funding in reporting year.

The effectiveness of the implementation of the State Program (subprogram) is recognized as high if the value of Egp (Ep/p) is not less than 0.9, average if the value of Egp (Ep/p) is not less than 0.8, satisfactory if the value of Egp (Ep/p) is not less than 0.7. In other cases, the efficiency of the implementation of the State programs (subprograms) are recognized as unsatisfactory (with the exception of recognition of the high level of impact of the risks identified in Chapter 6).